

SCHEDULE 2 – THE SERVICES

Service	Bariatric Surgery Follow Up Local Enhanced Service (LES)
Commissioner Lead	NHS Devon ICB
Provider Lead	General Practices in Devon
Period	01.11.2025 to 31.10.2028 (3 months' notice will be required to terminate this contract by either party)
Date of review	01.11.2027

1. Population and geography to be served

Obesity classification is typically based on a Body Mass Index (BMI) of 30 or above. In Devon 31% of the population are obese (based on the practices that have signed up to the One Devon Dataset, with a BMI recorded).

Bariatric surgery may be a treatment option for people with a BMI of 40 or over or a BMI over 35 with at least one comorbidity that is likely to improve with weight loss. In Devon 5% of the population has a recorded BMI of 40 or above, which equates to in the region of 17,300 patients.

Patients in Devon typically choose to have their NHS funded bariatric surgery at University Hospital Plymouth or Musgrove Park Hospital Taunton. Follow up for the first two years following surgery is provided by these specialist bariatric services. Following discharge from these services at two years post-surgery, lifelong follow up care is required to optimise patient outcomes and minimise the risk of complications.

Bariatric surgery follow up care shall be offered to adults aged >18 years from two years post-surgery onwards who have undergone NHS or privately funded bariatric surgery who are registered with a Devon GP within the Devon Integrated Care Board footprint.

2. Service aims

The aim of this LES is to provide structured, lifelong follow up for patients who have undergone bariatric surgery delivered by General Practice Providers, from two-years post-surgery.

This service will enhance patient outcomes by embedding bariatric follow up care into general practice, improving accessibility, continuity and quality of care.

3. Service outcomes

Service outcomes for bariatric follow up in general practice shall focus on the following:

- Improved clinical outcomes for post bariatric patients
- Early identification and management of complications
- Improved adherence to NICE NG246 (January 2025)
- Improved multidisciplinary communication and care coordination

See also section nine for the key performance indicators.

4. Service delivery

This service shall be delivered at individual practice or primary care network level.

5. Service description

In line with NICE NG246 and the British Obesity and Metabolic Surgery Society [BOMSS](#) guidance, following discharge two-years post-surgery from specialist bariatric services, this LES shall offer patients at least annual monitoring of nutritional status and appropriate supplementation after bariatric surgery, as part of a collaborative care model with specialist bariatric services.

This will offer a structured approach that ensures patients receive seamless high-quality care across both settings with roles and responsibilities as follows:

General practice

First point of contact

- Managing mild complications or concerns.

Routine nutritional management

- Provision of quarterly vitamin B12 injections.

Routine annual management and lifestyle support:

- Blood tests (in line with [BOMSS](#) guidance). These can be requested via the ICE electronic requesting and viewing system.
- Nutritional monitoring and supplements in line with [BOMSS](#) guidance.
- Monitoring for comorbidities and adjusting medications for these as required.
- Dietary and nutritional assessment, advice, and onward referral for support as required.
- Physical activity advice and onward referral for community support.
- Screening for mental health issues and onward referral for support as required.

Specialist bariatric services

- Provision of a post-surgery hospital discharge summary informing general practice of the surgery undertaken, post operative care, prescribed medication and follow up appointments.
- Provision of a care plan on discharge from the service outlining any patient goals, required interventions and follow up requirements.
- Advice and guidance via virtual MDTs for complex cases.
- Specialist follow up for complications via a rapid-access pathway back to the bariatric service when red flags arise.

See also appendix one for the patient pathway.

6. Criteria

All patients who have undergone the following surgical bariatric procedures shall be eligible for the follow up care as defined in this specification:

- Gastric sleeve
- Gastric band
- Gastric bypass

For patients who have privately funded bariatric surgery, standard follow up care post-two years surgery as detailed in this specification applies.

7. Prescribing

Supplementation

Supplementation is required life-long and needs to be taken by all patients to prevent any vitamin or mineral deficiencies developing. This should be provided in line with [North and East](#) Devon / [South and West](#) Devon local formulary and [BOMMS](#) guidance.

Post Bariatric Surgery Pregnancy

Pregnancy post bariatric surgery has increased risks for mother and baby. Guidance regarding post bariatric surgery pregnancy is available via [BOMSS](#) and on the [North and East](#) Devon / [South and West](#) Devon local formulary.

8. Payment, validation and key performance indicators

Payments

The table below provides the details of the full payments that can be expected for delivery of this LES, annual inflationary uplift will apply.

Bariatric procedure	Care intervention	Total
Gastric band	Annual review	£34.36
Gastric sleeve	Quarterly B12 injections and annual review	£61.08
Gastric bypass	Quarterly B12 injections and annual review	£61.08

Payment under this enhanced service will be a year-end payment based on provision of the above care interventions.

For individual practices who sign up to deliver the LES, annual claims for payment will be paid to individual practices upon submission of activity delivered.

For Primary Care Networks (PCN) who sign up to deliver the LES, the lead practice in the PCN will be contractually responsible for management of the service across all practices within the PCN who are delivering the activity.

Annual claims for payment will be paid directly to the lead practice upon submission of all activity delivered across the PCN. The lead practice is responsible for distributing the payment to each practice who has delivered the activity, in line with the rate specified in the table above and activity undertaken.

The ICB may undertake post payment verification including auditing claims to ensure that they meet the requirements of this enhanced specification.

Review Body on Doctors' and Dentists' Remuneration (DDRB) will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

9. Supporting guidance

Local

Supplementary documents have been produced to support with delivery of this enhanced specification. These are as follows:

- Bariatric follow up [supplementary guidelines](#) for both NHS and privately funded bariatric surgery.
- Patient [information leaflet](#) outlining the key considerations for people considering privately funding bariatric surgery.

National

[NICE NG246](#) Overweight and obesity management guideline.

[BOMSS](#) GP hub providing a range of guidance relating to bariatric surgery and associated follow up.

Key performance indicators

The following information shall be reported to NHS Devon ICB Commissioners on an annual basis in line with annual claims for payment.

Outcome	Requirements	Measurement
Improved clinical outcomes for post bariatric patients	80% of patients have blood tests for nutritional monitoring within the last 12 months 80% of patients have quarterly vitamin B12 injections	EMIS/S1 audit
Early identification and management of complications*	100% of abnormal blood results are proactively reviewed and actioned within eight weeks 100% of complications identified in general practice are either managed or referred to appropriate services within eight weeks of identification	Record of audit complications and action taken
Improved adherence to NICE NG246 (January 2025)	90% of patients have a documented offer of annual review (in line with NICE NG246 and BOMSS guidance)	Clinical record review
Improved multidisciplinary communication and care coordination	95% of referrals to secondary care include complete and relevant information	Audit of referral documentation

*Complications defined as: infection at a gastric band access port, sweating, dizziness or fainting after eating, difficulty swallowing and/or vomiting, abdominal pain, heart burn/reflux, diarrhoea, symptoms suggestive of mineral/vitamin deficiencies and psychological distress as detailed in [BOMSS](#) guidance for management of patients post bariatric surgery. Also see the [BOMSS](#) traffic light poster for the management of complications post bariatric surgery.

Appendix one - patient pathway

